FREE FINANCING REPORT

Medical Office & Healthcare Financing Report

Why This Sector Attracts Defensive Capital, What the 2026 Environment Looks Like, and the Metrics That Drive Approvals

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Why Sponsors Pursue Medical Office & Healthcare Financing

Medical office has emerged as one of commercial real estate's most defensive property types, and 2026 is reinforcing that reputation. That combination — demand driven by demographics rather than corporate cycles, a construction pipeline pulling back sharply, and cap rates compressing even as broader office struggles — is exactly why medical office and healthcare financing continues to attract broad, competitive capital.

- **Occupancy has held up far better than conventional office.** National medical office occupancy runs roughly 92% to 93%, with several submarkets in the mid-90s, a level general office has not seen in years.
- **New construction has pulled back sharply.** National MOB construction completions are projected to fall approximately 26% in 2026, the lowest level in over a decade, tightening supply just as demand keeps growing.
- **Cap rates are compressing, not widening.** Q1 2026 medical office cap rates averaged 6.9%, the first sub-7% reading since Q3 2024, signaling renewed investor confidence in the sector.
- **Rent growth remains positive and, in some markets, strong.** National average asking rent runs about $25.40 per square foot NNN, up 1.6% year-over-year, with faster-growing metros seeing gains well above that.
- **A durable, demographically driven demand story.** Roughly 11,400 Americans turn 65 every day, and industry forecasts point to about 18% growth in medical office demand volume over the next decade.

That said, lenders still underwrite tenant credit, lease term, and clinical functionality carefully. Weak or short-term tenancies, significant near-term rollover, and buildings poorly suited for clinical use continue to face tighter scrutiny regardless of the sector's overall strength.

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2026 Market Outlook for Medical Office & Healthcare Financing

Medical office enters 2026 as one of the more defensive corners of commercial real estate, with fundamentals and capital markets both moving in the sector's favor even as general office remains under pressure. The following figures outline where the market stands:

- National medical office vacancy stood at 9.8% as of Q4 2025, while occupancy across the top 100 metros runs approximately 92.7%, and direct vacancy for buildings 10,000 square feet and larger is notably tighter at 5.8%.

  Source: CBRE, Q1 2026 U.S. MOB Figures; Revista

- Rent growth remains positive: national average asking rent is $25.40 per square foot on a triple-net basis, up 1.6% year-over-year, with new construction commanding a premium at roughly $33.06 per square foot versus $24.78 for existing product.

  Source: CBRE, Q1 2026 U.S. MOB Figures

- New supply is contracting meaningfully: construction starts fell to just 1.0% of standing inventory at the Q4 2024 trough, under-construction inventory totals roughly 33.5 million square feet (about 2% of total stock), and national MOB completions are projected to fall approximately 26% in 2026 — the lowest level in over a decade.

  Source: CBRE, Q1 2026 U.S. MOB Figures; MMCG Invest, 2026 Outlook

- Investor demand is accelerating: Q1 2026 medical office cap rates averaged 6.9%, the first sub-7% reading since Q3 2024, on transaction volume of $2.9 billion for the quarter following $8.2 billion in Q4 2025.

  Source: CBRE, Q1 2026 U.S. MOB Figures

- Tenant retention remains a core strength of the sector: major healthcare REITs report retention rates ranging from 79% to 93.5%, with Healthcare Realty posting same-store cash NOI growth of 6.9% in Q1 2026.

  Source: CBRE, Q1 2026 U.S. MOB Figures

- Financing costs remain elevated relative to the sector's operating strength: mid-2026 CMBS coupon rates for medical office run roughly 6.5% to 7.75% depending on tenant credit and rollover risk, with maximum leverage of 60% to 70% loan-to-value, minimum DSCR of 1.25x to 1.35x, and debt yield typically clearing in the 9% to 10% range.

  Source: PeerSense, Medical Office CMBS Loans, July 2026

Bottom line for 2026: Medical office is trading and financing like the defensive asset class it has become — occupancy is well above general office, new supply is shrinking to decade-low levels, and cap rates are compressing rather than widening. But underwriting on the debt side remains disciplined: tenant credit, lease term, and clinical functionality still drive the difference between a smoothly financed deal and a difficult one, and financing costs remain elevated relative to where they were a few years ago.

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Critical Financing Metrics for Medical Office & Healthcare Properties

Lenders in this space underwrite tenant credit and lease structure as closely as the real estate itself. The following metrics most often determine whether a request moves forward:

1. **Tenant Credit Quality and Specialty Mix**
   The credit strength of health systems, physician groups, or credit-rated operators occupying the building. Health-system-anchored buildings generally command the best leverage and pricing.

2. **Remaining Lease Term and Rollover Exposure**
   The weighted average remaining lease term and the concentration of near-term expirations. Rollover-heavy buildings face tighter DSCR and leverage requirements.

3. **Occupancy Trend**
   Current occupancy relative to the market. Direct vacancy for larger buildings is notably tighter than the headline national figure. ~92%–93% national occupancy

4. **Clinical Functionality of the Space**
   Suitability for exam rooms, procedure space, imaging, power, and accessibility requirements. Generic office space repositioned for medical use without clear clinical demand faces added scrutiny.

5. **Rent-to-Market Comparison**
   In-place rents versus current market rents, relevant given the national average and stronger growth in select metros. ~$25.40/SF NNN national avg

6. **Debt Service Coverage Ratio (DSCR)**
   Net operating income measured against total annual debt service, varying by tenant anchor and rollover profile. 1.25x–1.35x typical minimum

7. **Loan-to-Value and Debt Yield**
   Leverage and income-to-debt cushion required by CMBS and portfolio lenders.
   60%–70% max LTV; 9%–10% debt yield

These metrics form the practical filter that separates medical office and healthcare financing requests that get funded from those that stall in underwriting.

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Practical Framework & Getting It Right the First Time

Pre-Submission Checklist

Before approaching lenders, confirm:

- [ ] Current rent roll with tenant names, specialties, lease terms, and square footage is organized
- [ ] Trailing 12-month operating statements are current
- [ ] Tenant credit information or practice financials for major occupants are prepared
- [ ] Property details relevant to clinical use (exam rooms, waiting areas, power, accessibility) are documented
- [ ] Market and comparable lease data for medical office are gathered
- [ ] Environmental reports (Phase I standard) are complete
- [ ] Existing debt schedule, entity documents, and organizational structure are ready
- [ ] Borrower/sponsor experience and financial information are current

Common Reasons Medical Office & Healthcare Financing Requests Underperform or Get Declined

- Missing or incomplete tenant credit details
- Weak or unclear lease-term and rollover information
- Incomplete operating history or financial statements
- Property poorly suited for clinical use without a documented repositioning plan
- Submitting to lenders unfamiliar with healthcare real estate underwriting

Final Perspective

Medical office financing in 2026 sits inside one of commercial real estate's genuinely strongest fundamental stories — occupancy well above general office, construction pulling back to decade-low levels, and cap rates compressing on renewed investor demand. But financing costs remain elevated, and lenders continue to underwrite tenant credit and lease structure closely regardless of how favorable the sector backdrop looks.

High-level knowledge of how these loans work is valuable. Turning that knowledge into a funded transaction still requires the right preparation and the right lending relationships.

Getting it right the first time matters. Missing tenant credit information, unclear rollover exposure, and mismatched lenders are among the most common reasons otherwise solid medical office financing requests lose time, money, and momentum.

YOUR NEXT STEP

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